

VYTALIX

Vytalix corporate presentation

Technology for Life · September 2026

THE GAP WE EXIST TO CLOSE

The gap we exist to close

- Copy
- Care fails most often between people.
- Between a woman and a changing set of professionals. Between one shift and the next. Between a community clinic and a hospital. Between a good idea and a safe, adopted product.
- Public inquiries in UK maternity care (Ockenden 2022; Kirkup 2022) repeatedly found failures to listen, escalate and hand over.
- Health-tech founders lose months to assurance and procurement they discover too late.
- Source line: reports named above, gov.uk publications; cite the current MBRRACE-UK report before using any disparity statistic.

Visual: Bone background; eyebrow "THE GAP" in Teal; two-column body; a simple monoline diagram of four "between" gaps as breaks in a path, with the Ember node marking the break. ---



Who we are

- Copy
- Vytalix is a UK health-technology group that advises, builds, delivers and teaches — so that technology earns its place in care.
- Founder-led. Pre-seed. UK home market, with active interest in maternal health in Nigeria and one further African market.
- Four pillars, one company: Advisory
- Consulting
- Health Solutions
- E-Learning.
- What we are not: NHS-approved, clinically validated, funded, or a team of twenty. Yet.

Visual: Bone; four pillar icons from the brand icon set (compass, spanner-and-path, Y-node shield, open book with signal line) in a row, each labelled; Y-node mark bottom-right. ---



FOUR PILLARS, ONE LOOP

Four pillars, one loop

- Copy
- Advisory informs Delivery. Delivery informs Product. Product informs Learning. Learning feeds Advisory.
- Advisory: independent, senior advice on strategy, market entry, regulation, funding readiness.
- Consulting: fixed-scope, fixed-price sprints — discovery, business cases, readiness reviews, market-entry playbooks, evaluation.
- Health Solutions: products designed for real care settings, starting with MaternaLink (in development).
- E-Learning: practical courses for health teams and founders (first courses month 9, ESTIMATE; no accreditation claimed).

Visual: a circular four-node monoline diagram, Teal strokes on Bone, Ember node at the centre labelled "care"; pillar labels in Manrope 700. ---



What we can do for you today

- Copy
- Advisory days and monthly retainers (from £1,200/day; £4,000/month for two days — ESTIMATE).
- Ten fixed-price consulting packages, £9,500–£28,000, two to six weeks each (ESTIMATE).
- Examples: Digital Health Discovery Sprint (2 weeks)
- DTAC & Evidence Readiness Sprint (3 weeks)
- Digital Maternity Readiness Review (4 weeks)
- Health Inequalities & Maternity Outcomes Review (5 weeks).
- Everything founder-led; associates only on signed work; no patient data processed in advisory or consulting.
- Full catalogue: ``/assets/SERVICE_CATALOGUE.md``.

Visual: Bone; three cards (Advisory / Consulting / Workshops) — cards are used because the viewer chooses between siblings (brand rule 6); prices in Manrope 700 with "ESTIMATE, ex-VAT" in Slate. ---



How we work

- Copy
- Fixed scope, fixed price, written change control.
- Evidence before claims: we cite the source or we don't use the number.
- Safety before speed: intended use, clinical safety (DCB0129/0160) and data protection come before features.
- Build with, not for: discovery interviews before design; "built with clinicians" only once true.
- We will tell you if your idea should not be built.

Visual: five short lines with monoline icons; generous whitespace; no imagery. ---



MATERNALINK (IN DEVELOPMENT)

MaternaLink (in development)

- Copy
- A care-coordination and communication platform for maternity services, designed to help women stay connected to their care team and help care teams hand over safely — for NHS and low-connectivity settings.
- v1 (PROPOSED): enrolment and consent
- contact schedules and reminders
- signed-off education in the woman's language
- secure two-way messaging with a named human owner
- a diary displayed, not interpreted
- care-team console
- structured handover summary
- escalation log

Visual: Night background section-style slide; product name with the "(in development)" tag rendered as part of the lockup; feature list in two columns; a persistent tag "Non-diagnostic — in development" top-right in Ember Light. ---



Roadmap with gates, not promises

- Copy
- Gates before anything is sold: (1) IP ownership documented
- (2) prototype audited for code ownership, security and data
- (3) Clinical Safety Officer appointed and hazard log opened
- (4) DPIA and lawful-basis analysis complete.
- v1 (12–16 weeks from funded start, ESTIMATE): non-diagnostic MVP; UK service evaluation; Nigeria SMS cohort with an implementing partner.
- v2 (months 7–15, PROPOSED): integration (FHIR, DHIS2 export), WhatsApp/USSD/voice, partner access; any monitoring feature requires a documented device-qualification decision first.
- v3 research track (month 15+, not a product commitment): the 2025 multimodal AI concept, advanced only with data partnerships, ethics approval, validation and a UK MDR / MHRA pathway.
- Detail:
 - ``/docs/04_MATERNALINK_PRODUCT_STRATEGY.md``,
 - ``/product/MATERNALINK_ROADMAP.md``.

Visual: horizontal waypoint diagram; four gate markers in Ember before the v1 waypoint; v3 drawn as a dotted branch labelled "research"; source line at 16px. ---



Where we work

- Copy
- United Kingdom: home and credibility market — services now; MaternaLink design partnership from month 6–18; paid pilots months 18–36 (`/docs/05_MARKET_STRATEGY.md` §8`).
- Nigeria: volume and impact market — one live conversation (status unknown); the play is partner, complement or move state, never displace on price; NGO- and donor-funded cohorts preferred until willingness to pay is validated.
- Kenya: third market for 2028+, positioned on facility coordination and referral.
- Everywhere else: consulting and e-learning only until 2028.

Visual: simple three-tier list with a small stylised map made of monoline contour fields (no national flags, no pins over cities). ---



Governance and safety posture

- Copy
- One English company (recommended "Vytalix Group Ltd", name TO VALIDATE) with the four pillars as divisions; founder holds 100% at incorporation; all IP assigned to the company.
- Board: founder plus one independent non-executive (PROPOSED). Advisory board of six profiles being recruited: obstetrics, midwifery/digital midwife, NHS digital and commissioning, Nigerian health-system leadership, regulatory and clinical safety, health-tech scaling. None appointed yet (KNOWN).
- Policies at incorporation: conflicts of interest, anti-bribery, data protection, clinical-claims sign-off.
- Insurance before the first paid engagement: professional indemnity and public liability (ESTIMATE ranges in `/docs/15\_LEGAL\_AND\_REGULATORY.md` §11).

Visual: two-column: "In place / In progress" — with dates in the bracketed fields [●]. ---



THE TEAM

The team

- Copy
- [Founder name], Founder — [two-line background: KNOWN facts only, confirmed against CV].
- Associates: engaged on signed work; named only with consent.
- Advisers: [name, role — only with written consent; otherwise "Recruiting: see slide 10"].
- Declared interests: the founder's separate venture, Kemek Enterprise Ltd (property finance), is declared and unrelated.

Visual: one portrait panel (illustration placeholder until real photography is commissioned with consent); roles in Manrope; no stock imagery. ---



What we are looking for / next step

- Copy
- Clients: a first conversation about one of the packages on slide 5 — a 30-minute discovery call, no charge.
- Partners: one UK maternity service to co-design MaternaLink (in development) as an unpaid service evaluation; one Nigerian implementing partner with an existing cohort.
- Advisers: the six profiles on slide 10.
- Investors: not raising today; building in public; SEIS/EIS advance assurance planned after incorporation (TO VALIDATE eligibility).
- Contact: [email]
- [LinkedIn]
- [website]

Visual: Deep background; wordmark; four short lines; Ember CTA button style for the contact line; footer "Technology for Life". --- ## Appendix slides (optional, not counted) - A1
Service catalogue table (from `/assets/SERVICE\_CATALOGUE.md` §3). - A2 MaternaLink inten

